

**Examining the Association Between Transportation and Access to Health Care in
Older Adults Living in Rural Saskatchewan**

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Poster Summary

In this research poster, the availability of alternative transportation methods in rural Saskatchewan for seniors is explored. The lack of alternative transportation is brought up as a factor preventing healthcare access for seniors, especially given that these communities have higher rates of chronic conditions that lead to seniors being unable to drive themselves. The main research question thus was whether socio-demographic circumstances and accessibility or lack thereof to alternative transportation impacts accessibility to healthcare for seniors in rural communities. The authors employed surveys to gather socio-demographic data and details about their access to alternative transportation methods or services and their access to health care services. They then looked for differences between those respondents who used and those who did not use alternative transportation methods in respect to socio-demographics, their usage of health care services, and the circumstances around their usage/non-usage of alternative transportation methods. The authors found that seniors who were socio-economically disadvantaged in that they tended to live alone and suffered from poorer health were more likely to be reliant on alternative transportation methods. However, most seniors who accessed health care services were healthier and enjoyed greater mobility, and would drive themselves to access health care services given the lack of alternatives in rural Saskatchewan. As such, a link was identified between inadequate and inaccessible transportation options and lack of accessibility to health care services for seniors. Research limitations included the lack of generalizability of these findings since most respondents were healthier and capable of driving themselves. Insufficient data was gathered for seniors who no longer drove and who used alternative transportation methods.

The Central Question: How to Improve Healthcare Access for Rural Seniors?

The unique challenges and experiences of seniors in rural areas is a well-documented area of concern. Rural areas struggle with a decline number of physicians, increasing wait times for accessing care, and less access to medical technologies compared to more populated areas. This means that seniors in these communities do not have easy access to such services like cardiac specialty care, and other such healthcare specialties that require more advanced technologies and infrastructure (Novak et al., 2022c). Given the lack of access to formal care, rural communities have resorted to setting up informal care and volunteer-run home care networks (Novak et al., 2022c). However, these networks often rely on younger people; and given the exodus of younger people from rural communities in search of better prospects in larger cities, these networks struggle to function effectively (Novak et al., 2022c). Several strategies have been proposed to alleviate these issues. The introduction of telehealth videoconferencing programs is one such potential solution. This healthcare delivery method provided high levels of satisfaction to both caregivers and patients suffering from memory-related neurological disorders in rural Saskatchewan (Novak et al., 2022b). This technology has seen significant adaptation; in 2018 alone one provider facilitated over one million consultations serving rural and remote Canadians (Novak et al., 2022c). Government incentives and quotas to facilitate physicians and nurses to take up work in rural areas have also been promoted as ways to bring accessible healthcare to such communities (Novak et al., 2022c).

Linking This Research to Course Material

The poster highlights the importance of getting rides to receive health care services where other forms of transportation are inaccessible. This ties in with the importance of strong social networks explored in the textbook, specifically where it talks about how one's friends can provide practical assistance in day-to-day life like providing transportation where needed (Novak

et al., 2022d). The poster also highlights that many seniors seeking alternative transportation are unable to do so due to the frequency of transportation to places with health care being insufficient, as well as the methods being themselves inaccessible for seniors, who have unique needs in this regard. These issues also hamper access to health care for seniors more generally regardless of geographic location; methods of transport such as long-distance buses and trains can be quite challenging for seniors to utilize. Getting to and waiting at stops for transportation can also be difficult for seniors, with part of the problem being the potential lack of frequency of arriving transport (Novak et al., 2022a). Of particular concern are Indigenous people given that many reside in isolated rural and northern communities. Where rides aren't offered or accepted, as seen in a case study exploring an Indigenous senior in Chapter 5, there often is no other alternative form of transportation available (Novak et al., 2022a). This can be especially problematic given that many Indigenous seniors do not have a good grasp of English, and thus may find available forms of alternative transportation inaccessible (Novak et al., 2022a).

Conclusion

In closing, urgent action is needed to relieve the inequities in health care access experienced by rural seniors across Canada. One option to incentivize community-based transportation assistance for seniors could be tax credits or subsidies to reimburse individuals who provide rides for seniors to health care providers in larger towns or cities. In order to reduce the need for seniors to use alternative transportation methods for accessing healthcare, a more widespread expansion of telehealth services to rural communities should be pursued; this would also reduce strain on the healthcare system and save money that could be invested elsewhere to serve seniors. These strategies and others like them will go a long way towards helping this often-overlooked marginalized demographic.

References

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